Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase III, Randomised, Double-blind, Placebo-controlled, Event-driven Study to Assess the Efficacy, Safety and Tolerability of Baxdrostat in Combination With Dapagliflozin Compared With Dapagliflozin Alone on Renal Outcomes and Cardiovascular Mortality in Participants With Chronic Kidney Disease and High Blood Pressure
Short Title:	A Phase III Renal Outcomes and Cardiovascular Mortality Study to Investigate the Efficacy and Safety of Baxdrostat in Combination With Dapagliflozin in Participants With Chronic Kidney Disease and High Blood Pressure
Protocol Number:	870388522340004598
NCT Number:	NCT06742723
Sponsor Name:	AstraZeneca
Investigational Product:	dapagliflozin
Phase:	PHASE3
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:

1. To determine whether baxdrostat/dapagliflozin is superior to placebo/dapagliflozin in reducing the risk of the omposite endpoint of ? 50% sustained decline in eGFR (estimated glomerular filtration rate), kidney failure or CV death.: Time to the first occurrence of any of the components of the composite endpoint of ? 50% sustained decline in eGFR (estimated glomerular filtration rate), kidney failure or CV death. 2. To determine whether baxdrostat/dapagliflozin is superior to placebo/dapagliflozin in reducing the risk of the composite endpoint of kidney failure: Time to the first occurrence of any of the components of the composite of:

? Onset of kidney failure:

* Sustained eGFR \< 15 mL/min/1.73 m2 or
* Chronic dialysis treatment or
* Receiving a kidney transplant or
* Death with a renal primary cause (death due to kidney failure when dialysis is not given) 3. To determine whether baxdrostat/dapagliflozin is superior to placebo/dapagliflozin in reducing the risk of the composite endpoint of CV (cardiovascular) death.: Time to the first occurrence of any of the components of the composite of CV (cardiovascular) death

Secondary Obj.:

1. To determine whether baxdrostat/dapagliflozin is superior to placebo/dapagliflozin at

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reducing UACR (urine albumin-creatinine ratio) after 16 weeks. 2. To determine whether baxdrostat/dapagliflozin is superior to placebo/dapagliflozin at reducing SBP (systolic blood pressure) after 16 weeks. 3. To determine whether baxdrostat/dapagliflozin is superior to placebo/dapagliflozin in reducing the risk of MACE (Major Adverse Cardiac Events).

2.2 Background & Rationale

International, Multicenter, Double-Blind, Placebo-Controlled and Event-driven study to assess efficacy, safety and Tolerability of Baxdrostat in combination with Dapagliflozin on renal outcomes and cardiovascular mortality in participants with chronic kidney disease and high blood pressure

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE3, Status: RECRUITING
Target N:	5000

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

1. Participants of any sex and gender must be ? 18 years of age at the time of signing the informed consent.
2. Participants with (a) or (b):

   1. eGFR 30-59 mL/min/1.73 m² (local or central laboratory value) AND:

      * UACR ? 30 mg/g (3.39 mg/mmol) and \< 500 mg/g (56.5 mg/mmol) (central laboratory value only), or
      * UACR ? 500 mg/g (56.5 mg/mmol) and ? 5000 mg/g (565 mg/mmol) (local or central laboratory value), or
      * UPCR ? 700 mg/g (79 mg/mmol) and ? 7000 mg/g (790 mg/mmol) (local laboratory value only).

   (b) eGFR 60-75 mL/min/1.73 m² (local or central laboratory value) AND:
      * UACR ? 500 mg/g (56.5 mg/mmol)) and ? 5000 mg/g (565 mg/mmol) (local or central laboratory value), or
      * UPCR ? 700 mg/g (79 mg/mmol) and ? 7000 mg/g (790 mg/mmol) (local laboratory value only)
3. \[obsolete\]
4. Participants with history of HTN and a SBP ? 130 mmHg (the most recent value within 4 weeks prior to screening or at the Screening Visit) and ? 120 mmHg at the Randomisation Visit.
5. Stable and maximum tolerated dose of an ACEi or an ARB (not both) for at least 4 weeks prior to Screening Visit.
6. Participants with:

   1. Serum or plasma potassium ? 3.0 and ? 4.8 mmol/L if eGFR ? 45 mL/min/1.73 m2 (local or central

      Exclusion Criteria:
      1. Systolic blood pressure \> 180 mmHg, or diastolic BP \> 110 mmHg at screening.
      2. Known hyperkalaemia, defined as potassium of ? 5.5 mmol/L within 3 months at screening.

3. Serum sodium \< 135 mmol/L (central or local laboratory values obtained within 4 weeks prior to screening or at the Screening Visit).

4\. Participants with T1DM will be excluded, except:

1. For US only: patients with T1DM treated with SGLT2i for at least 4 months, without DKA during that period, and who have experience with ketone monitoring are eligible for inclusion.
2. For Japan only: patients with T1DM treated with dapagliflozin 10 mg for at least 4 months, without DKA during the period of dapagliflozin treatment are eligible for inclusion.

5 Uncontrolled T2DM with HbA1c \> 10.5% (\> 91 mmol/mol) (central or local laboratory values obtained within 3 months prior to screening or at the Screening Visit).

6 New York Heart Association functional HF class IV at screening.

7 Stroke, transient ischaemic cerebral attack, valve implantation or valve replacement, carotid surgery, or carotid angioplasty, acute coronary syndrome, or hospitalisation for worsening heart failure within previous 3 months prior to randomisation.

8 Documented history of adrenal insufficiency.

9 Any dialysis (including for acute kidney injury) within 3 months prior to Screening Visit.

10 Any acute kidney injury within 3 months prior to the Screening Visit.

11 History of organ transplant or bone marrow transplant, or planned organ transplant within 6 months following randomisation (including kidney transplant).

12 Any clinical condition requiring systemic immunosuppression therapy other than maintenance therapy (stable for at least 3 months prior to Visit 1).

4.2 Exclusion Criteria

1. Systolic blood pressure \> 180 mmHg, or diastolic BP \> 110 mmHg at screening.
2. Known hyperkalaemia, defined as potassium of ? 5.5 mmol/L within 3 months at screening.
3. Serum sodium \< 135 mmol/L (central or local laboratory values obtained within 4 weeks prior to screening or at the Screening Visit).

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2. For Japan only: patients with T1DM treated with dapagliflozin 10 mg for at least 4 months, without DKA during the period of dapagliflozin treatment are eligible for inclusion.

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11. Study Identifiers & Governance

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Registry Number:	NCT06742723
Sponsor:	AstraZeneca
Study Title:	A Phase III Renal Outcomes and Cardiovascular Mortality Study to Investigate the Efficacy and Safety of Baxdrostat in Combination With Dapagliflozin in Participants With Chronic Kidney Disease and High Blood Pressure

15. Sign-off & Approval

Principal Investigator: ______________________ Date: ________

Clinical Operations Lead: ____________________ Date: ________

Lead Statistician: ___________________________ Date: ________